

Q I have reached my due date and my doctor is talking about inducing my labor. What does this mean?

Induction means artificially kick-starting labor, and if your baby is late it will be offered at 41 weeks. After 42 weeks the placenta becomes less efficient and the risk of stillbirth increases, so induction is often recommended before that point.

Your doctor will explain to you why it is appropriate to consider induction in your case, and the options available. Even if you are 41 or 42 weeks pregnant, you can request to “wait it out,” but the associated risks will be discussed so that you can make an informed decision about what’s best for you. If you decide you’re happy for intervention to get your labor started, she will begin by suggesting a cervical “sweep.” Your doctor can perform the sweep at a normal prenatal appointment; she will insert her index finger into your vagina and make a circular, sweeping

motion beyond your cervix, around the wall of your uterus, with the goal of separating the two membranes that surround the baby. This can provide the hormonal stimulation your body needs for your cervix to start softening and labor to start naturally within the next week or so. About half of all women who have a sweep will go into labor within two days. The procedure may be repeated if you haven’t gone into labor within a week. If a cervical sweep is unsuccessful, medical induction will be suggested.

MEDICAL INDUCTION METHODS

If a sweep didn’t work to start your labor, your doctor will explain the medical methods available and the reasons why they are appropriate for you (or not, as the case may be).

	WHAT IS IT?	WHEN AND HOW?	SUCCESS
Prostaglandin	Prostaglandin is a naturally occurring hormone that triggers the softening of the cervix. A gel or suppository containing the hormone is inserted the vagina, or you may be offered it in tablet form.	A doctor will administer the gel, suppository, or tablet at the hospital. You will be asked to stay there to be monitored, because some women have strong contractions after a short period of time, while others may need more than one dose. Be sure to bring along your packed hospital bag if your doctor tells you that you’re going to be induced.	Studies show that if your cervix was already softening, a prostaglandin induction can start labor within 24 hours of treatment.
Membrane rupture	Also known as ARM (artificial rupture of membranes) and amniotomy, this technique involves a doctor deliberately breaking your water to kick-start the labor process.	You’ll be offered this if you have not had a successful prostaglandin attempt. You will usually need to be in hospital because rupturing your membranes increases the risk of infection. Your doctor will use a special hook, called an amniohook, to puncture the amniotic sac in two places, in the lower part and in the upper part.	There is no guarantee of success. However, used with oxytocin, studies show that women will generally go into labor within 24 hours of membrane rupture.
Oxytocin	This is the hormone that your body produces when it feels your baby pressing down on your cervix, and it starts labor.	Oxytocin is the most common form of labor induction used in the US today. It’s most likely that it will be administered intravenously. Your doctor will suggest oxytocin if prostaglandin gels haven’t been successful.	If this last step in the induction process hasn’t worked you will be asked to consider a cesarean section.

Q I can suddenly breathe more easily—has my baby moved position? Is that a bad sign?

If you’re approaching your due date, it’s more likely to be a good sign. To prepare for birth, your baby makes a descent into the lowest part of your pelvis—this is called “lightening” (or sometimes “dropping”) because it “lightens” the pressure in your abdomen. The effect of your baby dropping into position is that your lungs suddenly have more space to expand, which means you can breathe more easily again.

On the flip side, you might find that the new position puts more pressure on your bladder, and you need the bathroom more often.

Q I’ve heard an induced labor can be more painful. Is this true?

When you go into natural labor, signals to your brain mean that your body releases endorphins, hormonal painkillers that help you to cope with labor and birth. In an induced labor, your body hasn’t had the natural gearing up that makes

sure you have good levels of circulating endorphins from the earliest, imperceptible beginnings of labor. As a result, many women report that an induced labor is more painful than one that occurs naturally. However, try not to worry—being induced doesn’t place any restrictions on the pain relief that is available to you, so ask your doctor about the options if you feel things are getting to be too much for you. Statistics show that women having an induced labor are more likely to ask for an epidural. Your baby is also slightly more likely (about 15 percent more likely) to need an assisted birth (see p.222).